

2000

BEFORE THE IOWA BOARD OF SOCIAL WORK EXAMINERS

IN THE MATTER OF:	)	
	)	DIA NO. 00DPHSW001
	)	
DEBORAH C. GUFFY	)	FINDINGS OF FACT,
Respondent	)	CONCLUSIONS OF LAW,
	)	DECISION AND ORDER

On March 11, 2000, the Iowa Board of Social Work Examiners (Board) filed a Statement of Charges against Deborah C. Guffy (Respondent) alleging that she had violated Iowa Code section 147.55(3) and 645 IAC 280.212(3), by engaging in unethical conduct in the practice of social work by engaging in a dual relationship with a patient or client (Count I) and 645 IAC 280.213(3)"f", by exercising undue influence on a patient or client (Count II). A hearing was scheduled for May 8, 2000. The hearing was rescheduled three times.

The hearing was held on September 25 and September 26, 2000 in the fifth floor conference room, Lucas State Office Building, Des Moines, Iowa. The Respondent appeared and was represented by her counsel, Connie Diekema. The state of Iowa was represented by Heather Adams, Assistant Attorney General. The following members of the Board were present for the hearing: Jennifer Lock Oman, L.I.S.W.; Mark Smith, L.I.S.W.; Janice McCoy, L.B.S.W.; and Joan Hester, Public Member. Margaret LaMarche, Administrative Law Judge from the Iowa Department of Inspections and Appeals, assisted the Board in conducting the hearing. The hearing was closed to the public, at the Respondent's request, pursuant to Iowa Code section 272C.6(1).

The Board convened in closed executive session, pursuant to Iowa Code section 21.5(1)(f)(1999), to deliberate its decision. Additional deliberations were conducted on December 11, 2000, and a motion was taken in open session. The administrative law judge was instructed to complete preparation of the written decision of the Board, in conformance with the directions of the Board.

THE RECORD

The record includes the Statement of Charges, Notice of Hearing, Answer; Continuance Orders, Orders Rescheduling the Hearing, the testimony of the witnesses, Respondent's Motion To Exclude Hearsay Statements, and the following exhibits:

- State Exhibit A: Statement of Charges, 3/11/00
- State Exhibit B: Answer, 3/30/00
- State Exhibit C: Investigative Report, 11/29/99

State Exhibit D: Patient Records, patient #1
State Exhibit E: Billing Records, patient #1
State Exhibit F: Patient Records, patient #2
State Exhibit G: Billing Records, patient #2
State Exhibit H: Statements, 10/7/97
State Exhibit I: Check #2341, 5/3/98
State Exhibit J: Receipts
State Exhibit K: Curriculum Vitae, Patricia Kelley, Ph.D.
State Exhibit L: Deposition Transcript, 5/2/00
State Exhibit M: Code of Ethics of the National Association of Social Workers
State Exhibit N: Complaint of patient #1, filed 3/24/99

Respondent Exhibit 1: Respondent Curriculum Vitae
Respondent Exhibit 2: Medical Records, patient #1
Respondent Exhibit 3: Medical Records, patient #1
Respondent Exhibit 4: Mental Health Center Records, patient #1 and patient #2
Respondent Exhibit 5: Medical Records, patient #1 and patient #2
Respondent Exhibit 6: Letter dated 5/16/00 and billing
Respondent Exhibit 7: dates of service, patient #1
Respondent Exhibit 8: dates of service, patient #2
Respondent Exhibit 9: Des Moines Police Report, 3/9/99
Respondent Exhibit 10: Johnston Police Report, 3/10/99
Respondent Exhibit 11: Letter dated 10/25/99 (Richard Guffy to whom it may concern)
Respondent Exhibit 12: Letter dated 4/26/00 (patient #2 to whom it may concern)

FINDINGS OF FACT

1. On July 16, 1991, the Respondent was issued license number 304-00741 by the Board to practice as a social worker, subject to the laws of the state of Iowa and the rules of the Board. (Testimony of Respondent; State Exhibit A)

2. The following time line is presented as an aid to understanding the Board's findings of fact:

3/1/96-10/7/97: Patient #1 has 23 therapy sessions with the Respondent; her husband (Patient #2) is present for 13 of the 23 sessions.

10/7/97: Boiler is purchased

10/28/97: Patient #1 is a no show for her scheduled appointment with the Respondent

1/15/98-4/21/98: Patient #2 has six individual counseling sessions with the Respondent

4/30/98: Patient #2 writes a letter to the Respondent, thanking her for her services and telling her that he knows if he needs services in the future, she will refer him.

5/3/98: Power washing services are purchased

8/9/98: Patient #2 begins providing remodeling services in the Respondent's home

9/24/98: Respondent gives letters to both Patient #1 and Patient #2 removing herself from further professional therapeutic relationships with them.

1/99: Both the Respondent and Patient #2 separate from their spouses.

Social work services for patient #1: 3/1/96-10/7/97

3. The Respondent began providing social work services for patient #1 on March 1, 1996, following a referral from the patient's family physician. Patient #1 was referred for treatment of anxiety and panic attacks. The Respondent's initial assessment of patient #1 did not include any diagnosis on Axis I or Axis II of the DSM-IV. The Respondent's treatment plan/treatment goals were to decrease active anxiety attacks, decrease depressive symptoms, to help gain insight into where all the stressors are coming from, and to develop new coping mechanisms to work with these. Patient #1 was also referred to a psychiatrist for an evaluation for medication.

In her initial assessment of patient #1, the Respondent noted that the patient's husband is a workaholic. The Respondent also noted that Patient #1 reported her marriage is good and they are committed, but she finds that she reacts to the stresses that her husband is under. Most of her friends had reported to her that they

were concerned that her husband was not enjoying life. (Testimony of Patient #1; Respondent; State Exhibit D)

4. The Respondent had a second therapy session with patient #1 on March 5, 1996. The Respondent's progress note states that the patient's husband's anxiety is contributing to her own. The Respondent and patient #1 discussed some changes she could make at home to decrease her anxiety with her husband and gave some specific areas that he could work on to help her.

The next session with patient #1 was March 13, 1996. At the end of this session, patient #1 indicated that her husband was willing to see the Respondent for one session, and the Respondent suggested that patient #1 bring her husband in for the next session. At the hearing, the Respondent testified that since patient #1 reported that her anxiety attacks were being caused by her husband, the Respondent had him come in as a "collateral" to give the Respondent insight into patient #1's problems.

The Respondent defined a "collateral" as someone who is brought into a therapy session because they are helpful to what is going on with the patient. She emphasized that the collateral is not the patient. She testified that if the collateral expressed a need for treatment she would need to refer the collateral to another therapist. (Testimony of Patient #1; Respondent; State Exhibit D)

5. On March 25, 1996, the husband of patient #1 accompanied her to her therapy session with the Respondent. Hereinafter, he will be referred to as patient #2. The progress note reflects that patient #2 came in at the request of the Respondent. Patient #1 and Patient #2 had been married for twenty-five years. Patient #2 told the Respondent about the immense stressors that he was experiencing, including family, business, and financial problems. Patient #2 also mentioned that the couples' five year old child was still sleeping in bed between them. They discussed the stressors in the marital situation and how everyone calls on patient #2 to handle everything. It was decided that patient #2 would think hard about what the Respondent said and see if they could start coming up with some new solutions to decrease the extreme stressors in the household. (Testimony of Patient #1; Respondent; State Exhibit D)

6. Patient #2 returned with patient #1 for the next appointment on April 15, 1996. Patient #1 had additional appointments with the Respondent through October 7, 1997. Patient #2 was present for thirteen (13) of the twenty-three (23) therapy sessions that patient #1 had with the Respondent. Discussions in the sessions where patient #2 was present included the tension level between the two of them, the fact that their son was still sleeping in their

bed and was in the middle of their marriage, dates that they had with each other, their patterns of communication, and problems in patient #2's family business. At a therapy session on August 1, 1996, the Respondent gave them a homework assignment to go out on a date and think about ways to improve their sex life. (Testimony of Patient #1; Respondent; State Exhibit D)

7. Patient #1 testified that her life was her marriage and her son, and she discussed her marriage with the Respondent at the therapy sessions. When her husband was present at the sessions, they discussed communication issues and their sex life. As the sessions with her husband continued, Patient #1 began to feel that they were spending more time discussing her husband's business problems and issues than her own problems.

After her session on October 7, 1997, Patient #1 decided to discontinue her therapy with the Respondent, because she felt that she was no longer benefitting from it. She told her husband why she was discontinuing therapy. On October 27, 1997, Patient #1 canceled her appointment with the Respondent for the following day and did not reschedule. She did not provide a reason for the cancellation. (Testimony of Patient #1; Patient #2; Respondent; State Exhibit D)

8. The Respondent denies that she provided marital counseling to patient #1 and patient #2. The Respondent testified that she rarely provides marital counseling. When she does, it is set up on a cash for services basis and both the husband and wife fill out fee agreements and Beck Depression scales. In this case, only patient #1 had filled out a Beck Depression scale. The bills for patient #1's therapy were submitted to her insurance company for reimbursement. (Testimony of Respondent)

Purchase of Boiler

9. The Respondent's ex-husband, Richard Guffy, is in the insurance business. Sometime in 1997, while he was still married to the Respondent, patient #2 came to him to request an insurance bid for his family business. Richard Guffy cannot recall the exact date, but recalls that they had two meetings, and he provided an insurance quote at the second meeting. During these meetings, patient #2 did not tell Richard Guffy that his wife had been in therapy with the Respondent.

In the summer of 1997, the Respondent and Richard Guffy purchased an older home. They later discovered that its existing boiler was dangerous and needed to be replaced. The Respondent agrees she may

have mentioned this to patient #1 and patient #2 in casual conversation just prior to a therapy session. The Guffys' insurance agent gave them the name of a person who could install a new boiler for them. The installer told them that there were really only two sources in Des Moines for purchasing this type of boiler. One of the sources was the family business of Patient #2. (Testimony of Richard Guffy; Respondent; Patient #2; Respondent Exhibit 11)

10. The Respondent told her husband that he should take care of the purchase and installation of the boiler. She was uncomfortable with the purchase given her relationship with Patient #1 and Patient #2. She could not tell her husband that she had provided social work services to Patient #1 or Patient #2 because this would be a breach of confidentiality. The Respondent felt that even if she just told her husband to buy the boiler somewhere else, without telling him why, he would realize the reason and she would have breached patient confidentiality.

At the hearing, Richard Guffy testified that Patient #2 told him "very early" that his wife and he had participated in therapy sessions with the Respondent. However, both the Respondent and Richard Guffy testified that they did not discuss this with each other. (Testimony of Respondent; Richard Guffy)

11. Richard Guffy purchased the boiler from Patient #2 at a price of \$1452.33. He felt that he got a good price on the boiler, but did not think that he got a deal that was not available to any member of the public. One of the invoices has both the Respondent and her husband's name on it; another invoice refers only to Richard Guffy. The Respondent was out of town when the boiler was delivered. (Testimony of Patient #2; Richard Guffy; Respondent; State Exhibit H; Respondent Exhibits 11, 12)

Social Work Services for Patient #2

12. In January 1998, Patient #2 contacted the Respondent and told her that he wanted to have individual therapy to assist him in dealing with stress connected with the sale of his family business. The first date of service for Patient #2 was January 15, 1998. The Respondent provided social work services to Patient #2 on five additional dates. The last appointment was April 21, 1998.

The Respondent's final session notes state that Patient #2's stress level was still high due to the company not being sold yet. The notes also state that Patient #2 thanked the Respondent for "her help in the process." Regarding a return visit, the Respondent's

notes state "will call if necessary." On April 30, 1998, Patient #2 wrote a letter to the Respondent thanking her for her services. The letter refers to having met her husband and socializing with him. In his last sentence, Patient #2 states:

I feel if ever I need more professional therapy, you would refer me to someone you feel can provide me with the proper therapy.

(Testimony of Respondent; Patient #2; State Exhibits F, G; Respondent Exhibit 8)

13. The Respondent testified that before she agreed to provide social work services to Patient #2, she called Patient #1 and asked her permission. According to the Respondent, she told Patient #1 that initiation of therapy with Patient #2 could affect their relationship, and she wanted her to be aware of that. The Respondent further stated that she told Patient #1 that if she had any hesitancy, she would refer Patient #2 to another therapist. According to the Respondent, Patient #1 stated that she was feeling great and did not think that she would need the Respondent's services in the future. She felt her husband would benefit from therapy and thought he needed the Respondent. The Respondent made no note of this telephone conversation in either the records of Patient #1 or Patient #2.

Patient #2 also testified that Patient #1 knew that he was receiving social work services from the Respondent, and that the Respondent told him that she would have to speak to Patient #1 before he started therapy.

Patient #1, however, insisted that she did not know her husband was seeing the Respondent for individual therapy. She testified that the first time she was aware of this was when her son was in the hospital in May 1998 following a trampoline accident. The Respondent came to the hospital with gifts for her son and gave her and her husband hugs. According to Patient #1, she was very surprised and asked her husband what the Respondent was doing there. She testified that this was the first time she knew that her husband had been seeing the Respondent for individual therapy.

The Board had difficulty resolving whether the Respondent in fact spoke to patient #1 and received permission before providing therapy to patient #2. All of the witnesses were emotionally involved and angry at each other, which could affect their credibility. However, if the Respondent had such a conversation with patient #1, it should have been documented in the patient's

file. The absence of any documentation by the Respondent persuades the Board to give greater weight to the testimony of patient #1, and to conclude that patient #1 was not consulted by the Respondent before services were initiated with patient #2. (Testimony of Respondent; Patient #1; Patient #2; State Exhibits D; F)

Purchase of Power Washing Services

14. On May 3, 1998, Patient #2 provided power washing services at the home of the Respondent and her husband, for a fee of \$221.00. The Respondent's husband paid for these services with a check from their joint checking account.

Patient #1 testified that during one of her therapy sessions, the Respondent told patient #2 that she and her husband were thinking of painting their house and asked if he would consider power washing it first. The Respondent admitted that patient #2's decision to purchase a power washer had been discussed in patient #1's therapy sessions. According to the Respondent, patient #1 wanted patient #2 to make more money, but the power washer would be an expensive purchase for patient #2 and providing power washing services would require him to spend even more time away from his family. However, the Respondent denied that she had ever discussed purchasing power washing services with patient #2. She testified that her husband handled all of the arrangements for the power washing, and she did not even realize that he had done so until the morning the services were provided. According to the Respondent, she was very upset that her husband had hired patient #2 to power wash their house. (Testimony of Patient #1; Patient #2; Respondent; State Exhibit I)

15. Richard Guffy testified that he made all of the arrangements with patient #2 to power wash the house and confirmed that the Respondent did not know about these arrangements until the day that the services were provided. (Testimony of Richard Guffy; Respondent Exhibit 11)

Patient #2 Hired for Remodeling Project

16. In June 1998, Richard Guffy began talking to various contractors regarding a kitchen remodeling project at his home. Richard Guffy wanted to work with any contractor that he hired, in order to help keep costs down. The Respondent knew that one of the persons that her husband was talking to about the remodeling project was patient #2. According to the Respondent, she called patient #1 before her husband hired patient #2 to work on the remodeling project. She believes that this phone call was made in

July 1998; she did not document it. The Respondent testified that she wanted to be sure that it would be all right with patient #1 if her husband was working evenings and weekends on the project. The Respondent testified that patient #1 told her that she had no problem with it.

It was anticipated that the project would take two to three months. In order to earn extra income, Patient #2 occasionally worked evenings and weekends for Steve Ely, a contractor. Richard Guffy and patient #2 had different recollections of their business arrangement. Richard Guffy thought that he initially hired Steve Ely, who employed patient #2 to do some of the work. He testified that eventually he just paid patient #2 directly for his time. Patient #2, however, testified that he bid on the job personally but suggested that Richard Guffy use Steve Ely as a consultant.

In either event, by early August 1998, patient #2 was spending long hours working in the Respondent's home on the remodeling project. The first invoice in this record, which is made out to Richard Guffy and the Respondent, shows a starting date of August 9, 1998. On August 25, 1998, patient #2 was paid \$670.00 for 33½ hours of labor at \$20.00 per hour.' (Testimony of Richard Guffy; Respondent; Patient #2; State Exhibit J)

17. In August 1998, patient #1 and patient #2 had a pool added at their home. Patient #2 was supposed to build a deck for the pool but was having difficulty finding time to work on it due to the long hours he was spending working at the Respondent's home. At some point, patient #2 suggested to Richard Guffy that he help him with his deck at home in exchange for some of his remodeling services. Richard Guffy and the Respondent both went to the home of patient #1 and #2 on at least two occasions to help work on the deck. On a few occasions, one of the Respondent's children was invited to swim at the pool. On two occasions, the Respondent and her husband went out to dinner with patient #1 and patient #2.

On one of the occasions when the Respondent was working on the deck, she delivered letters to patients #1 and #2. The letters, dated September 24, 1998, were essentially identical. In the letters, the Respondent states that the letter "is a formal process that I am required to have documented in your chart." She further states:

The changes in our associations negate my ability to provide objective professional guidance and insight as I have in our past relationship. Do (sic) to these changes, I need to remove myself from any further therapeutic relationships with you.

I will, however, always be willing to give recommendations for a proficient psychotherapist in the future if the need arises...

(Testimony of Patient #1; Patient #2; Respondent; Richard Guffy; State Exhibit D)

18. Based on the paid invoices, work on the kitchen remodeling continued from August 1998 until at least February 14, 1999. The work was primarily performed during the evenings and weekends since everyone involved in the project had other jobs. While patient #2 and the other workers usually finished for the night by 9:00 p.m., they would occasionally work as late as midnight to complete a specific task. Patient #2 spent a lot of nights and weekends at the Respondent's home, including some 10 or 10½ hour days. Richard Guffy usually worked alongside the workers he had hired. Richard Guffy testified that there were a few occasions when he was not present during the remodeling because he had to leave town to visit his mother, who was seriously ill.

The Respondent primarily cared for the couple's two children during the evenings and weekends and had only limited direct involvement in the remodeling project. She may have occasionally held a board for one of the workers or accompanied someone to pick up supplies if a check had to be written. On one occasion, patient #1 called the Respondent's home to speak to her husband and was told that he and the Respondent had gone to Menard's together. (Testimony of Respondent; Richard Guffy; Patient #2; State's Exhibit J)

19. Patient #1 became upset by the many nights and weekends her husband spent remodeling the Respondent's home. She repeatedly asked him when the project would be finished and repeatedly asked him to subcontract some of the work, reminding him that his family needed him at home. Patient #1 felt that her relationship with her husband changed, that he became bitter and angry and they were fighting all the time. If she was ever critical of the Respondent, patient #2 immediately defended her.

According to patient #1, on one occasion patient #2 did not come home until 2:00 a.m. When she confronted him about this, he told her that he had just received four hours of free therapy. The Respondent, Richard Guffy and patient #2 all deny that any therapy services were ever provided at the Guffy home. The Board was unable to conclude, by a preponderance of evidence, that the Respondent provided therapy to the patient #2 in her home on this occasion. (Testimony of Patient #1, Patient #2; Richard Guffy; Respondent)

20. On another occasion in late October or early November 1998, patient #1 called the Guffy home to speak to her husband, and the Respondent's son answered. He told patient #1 that her husband could not come to the phone because he was being hypnotized. Patient #1 became very angry, and the Respondent's son reported the telephone conversation to his mother.

In fact, the Respondent was hypnotizing patient #2 on this occasion. Patient #2 had been trying to stop a tobacco chewing habit and had asked the Respondent for her help on several occasions. He knew that she had hypnotized people in the past to stop smoking or stop overeating. The Respondent received training in hypnosis from the American Society of Clinical Hypnosis. The Respondent used hypnosis only rarely in therapy sessions. The Respondent described her hypnosis of patient #2, like the hypnosis she sometimes did for friends, as being just "for fun." After patient #1 called, the hypnosis session was not completed. (Testimony of Respondent; Patient #1; Patient #2; Respondent Exhibit 1)

21. In January 1999, patient #2 told patient #1 that he was moving out and wanted a divorce. Patient #2 moved in with his mother, but had two dogs which could not stay in his mother's home. Patient #2 asked the Respondent and Richard Guffy if he could build a kennel for his dogs on their garage. They agreed to this, and patient #2 moved his dogs to the Respondent's yard.

Within one month of the separation of patient #1 and patient #2, the Respondent and her husband also separated. Initially, the Respondent and patient #2 were using the same lawyers. The Respondent testified that she changed lawyers when she realized this. Patient #1 feels that the Respondent was the cause of her marital dissolution. Patient #2, the Respondent and Richard Guffy all deny that the Respondent was a factor in the dissolution of the marriage of patient #2 or that patient #2 was a factor in the dissolution of the Respondent's marriage. (Testimony of Respondent; Patient #1; Patient #2; Richard Guffy)

22. Shortly after patient #2 moved his dogs to the Respondent's home, his older dog became very ill. Patient #2 spent one night sleeping with his dog on the floor at the Respondent's home. The following day, the Respondent accompanied patient #2 to the vet to put the dog to sleep. The receptionist at the veterinarian's office reported this to patient #1 and also told her that the Respondent and patient #2 appeared to be a couple. (Testimony of Patient #1; Patient #2; Respondent)

23. On March 9, 1999, a friend of patient #1 told her that the Respondent was also filing for divorce. Patient #1 was extremely angry and called the Respondent's house. Patient #1 asked the Respondent "how long have you been screwing my husband?" and told her that she better "watch her job." The Respondent became very frightened by the telephone call and called the police to report it.

The police talked to patient #1 the following day and warned her not to make any further telephone calls. No charges were filed. However, later that day patient #1 went to her mother-in-law's home intending to leave a note for her husband. When she entered his room and saw a photograph of their son, she became extremely angry. She tore up the photograph and vandalized the room. The police were called, and patient #1 was later charged with misdemeanor criminal mischief, to which she pled guilty. (Testimony of Respondent; Patient #1; Patient #2; Respondent Exhibits 9, 10)

24. Patient #1 was told by her son and others that Patient #2 and the Respondent were spending a lot of time together. She was told that they would spend time together with their children, going to movies and out for meals. Her son told her that he had spent the night at the Respondent's home with his father. In addition to the night he spent at the Respondent's with his sick dog, patient #2 admitted that he fell asleep on the couch in the Respondent's family room while watching movies with the Respondent and her children.

Patient #1 also learned that the Respondent accompanied patient #2 to his high school reunion in June 1999 and to a family reunion in August 1999. The Respondent spent the night at the home of patient #2's mother on one occasion, when she and son were sick with the flu. Patient #2 continues to keep his dog at the Respondent's home and goes there once or twice a day to care for the dog. Patient #2 often parks his trailer at the Respondent's home and keeps a broken riding lawn mower there as well. Patient #1 is convinced that there is a romantic relationship between her ex-husband and the Respondent. She testified that as a result of this experience she has lost her trust in therapists. (Testimony of Patient #1; Patient #2; Respondent)

25. The Respondent and patient #2 admit that they have a close friendship but deny that it has ever been a romantic or sexual relationship. The Respondent denies any issues of transference or counter-transference between herself and patient #2. In the Respondent's view, her patient-therapist relationship with patient #1 was terminated on the day that she called patient #1 to see if she objected to her serving as therapist for patient #2. The

Respondent believes that her patient-therapist relationship with patient #2 terminated on April 30, 1998, when he wrote her the letter thanking her for her services and expressing confidence that she would refer him in the future if he needed additional counseling. (Testimony of Respondent; Patient #2)

26. The Respondent does not believe that her actions have caused harm to either patient #1 or patient #2. The Respondent, two of her colleagues, and two patients testified generally about her usual conservative approach to potential dual relationships or conflicts of interest. In general, the Respondent strictly avoids social relationships with patients and former patients. (Testimony of Respondent; Ernestine Griswold; Donald Heywood; two patients)

27. The Code of Ethics for the National Association of Social Workers (NASW) is applicable to all social workers. The Respondent is a member of NASW. The NASW Code of Ethics provides, in relevant part:

1.06 Conflicts of Interest

(a) Social workers should be alert to and avoid conflicts of interest that interfere with the exercise of professional discretion and impartial judgment. Social workers should inform clients when a real or potential conflict of interest arises and take reasonable steps to resolve the issue in a manner that makes the clients' interests primary and protects clients' interests to the greatest extent possible. In some cases, protecting clients' interests may require termination of the professional relationship with proper referral of the client.

...

(c) Social workers should not engage in dual or multiple relationships with clients or former clients in which there is a risk of exploitation or potential harm to the client. In instances when dual or multiple relationships are unavoidable, social workers should take steps to protect clients and are responsible for setting clear, appropriate and culturally sensitive boundaries. (Dual or multiple relationships occur when social workers relate to clients in more than one relationship, whether professional, social or business. Dual or multiple relationship can occur simultaneously or consecutively).

(State Exhibit M; NASW, Code of Ethics, 1.06)

28. Patricia L. Kelley, Ph.D., is a professor at the school of Social Work at the University of Iowa and is a licensed social worker in Iowa. Dr. Kelley reviewed the record in this case and offered the following testimony as an expert witness:

a) Dual Relationships

A dual relationship is when the social worker develops a secondary relationship with a client, such as a business or social relationship. The purchase of goods or services from a client is a business relationship. According to Dr. Kelley, such a relationship with a client "muddies the water." There is a power relationship between the social worker and client. If the client is the offeror of goods or services, the client may not feel comfortable charging their customary fee. Additional problems can arise if there is dissatisfaction with the goods or services. Contracting with a client to provide remodeling services in the social worker's home is especially problematic because the client is present in the social worker's home, observing the social worker's private life and family interaction.

b) Scope of Client-Therapist Relationship

In general, the client's whole family system is encompassed in the patient-therapist relationship. Therefore, the client-therapist relationship extends to other members of the family when the member is being treated.

This is particularly true under the circumstances presented by the facts in this case. Patient #1 initiated therapy with the Respondent, but subsequently her husband (patient #2) was brought in to participate in 13 of the 23 therapy sessions with patient #1. Topics of discussion in the therapy sessions included stressors on patient #1 due to her husband's work and long hours away from the family, issues relating to the couple's young child, communication issues between husband and wife and intimate details of the marital relationship. Regardless of whether this was or was not marital counseling, as a result of these sessions the Respondent's duties as a therapist extended both to patient #1 and patient #2.

c) Termination of the Client-Therapist Relationship/Subsequent Relationships With Client

It is unethical to terminate a client-therapist relationship in order to pursue another relationship with a client. The power relationship between the client and therapist continues beyond the termination of the relationship. There is a difference of opinion concerning how long a social worker must wait before initiating

other relationships with a former client, although Dr. Kelley testified that a sexual relationship is never appropriate.

In Dr. Kelley's opinion, it was clearly inappropriate for the Respondent to enter into a business relationship with patient #2 in July or August, even if the client-therapist relationship was terminated on April 30 as asserted by the Respondent. Moreover, the relationship quickly became a very close social relationship, with the Respondent and patient #2 working at each other's homes, going out to dinner, attending reunions, etc. Dr. Kelley testified that one of the potential dangers of such relationships is that the therapist knows too much about the client to have an equal friendship or relationship. Moreover, the Respondent owed a duty to patient #1, who was harmed as a result of the relationship between the Respondent and patient #2. Social workers are obligated to do no harm to their clients. The needs of the client must supercede the needs of the social worker.

d) Documentation

Dr. Kelley testified that all phone conversations with clients should be recorded in the client's record. Termination of the client-therapist relationship should also be recorded in the therapist's notes. The Respondent did not document the termination of her professional relationship with these patients in her notes until at least September 24, 1998, when she included a copy of her termination letter in their records. (Testimony of Patricia Kelley, Ph.D.; State's Exhibits D, K)

CONCLUSIONS OF LAW

I. COUNT I

Count I charges the Respondent with engaging in unethical conduct in the practice of social work by engaging in a dual relationship with a patient or client.

Iowa Code section 147.55(3) (1997) provides, in relevant part:

147.55 Grounds.

A license to practice a profession shall be revoked or suspended when the licensee is guilty of any of the following acts or offenses:

...

3. ...engaging in unethical conduct or practice harmful or detrimental to the public. Proof of actual injury need not be established.

645 IAC 280.212(3) provides that the board may impose any of the disciplinary sanctions provided in rule 280.211 when the board determines that the licensee is guilty of engaging in unethical conduct or practice harmful or detrimental to the public. Proof of actual injury need not be established.

The preponderance of the evidence established that the Respondent engaged in inappropriate dual relationships with patient #2, which constituted unethical conduct, pursuant to Iowa Code section 147.55(3)(1997) and 645 IAC 280.212(3).

A. Respondent's Ethical Duty Extended To Both Patients

From the time that therapy was initiated with patient #1, the Respondent had an ethical duty to avoid dual relationships with patient #1 or her husband (patient #2). Patient #2 belonged to the family system of patient #1. Patient #2 actively participated in 13 of patient #1's 23 therapy sessions. Many of the topics discussed in therapy directly concerned patient #2 and his relationship with patient #1. The Board does not accept the Respondent's characterization of patient #2 as only a "collateral" to the therapy of patient #1 prior to January 15, 1998. On January 15, 1998, patient #2 began individual therapy with the Respondent.

B. Termination of the Patient-Therapist Relationship

This Board has previously held that the termination of the patient-therapist relationship must be recorded in the therapist's notes. Without such a firm delineation of the end of the relationship, it cannot be said to have ended. In the Matter of Robert Thomas McTaggart, DIA No. 96DPHSW001, issued March 14, 1997. This principle was reiterated by Dr. Kelley in her testimony.

1. Patient #1

There was no documentation of the termination of the patient-therapist relationship with patient #1 until September 24, 1998, when the Respondent gave patient #1 a termination letter and included it in the patient's record. The Respondent's notes for her last session with patient #1 on October 7, 1997 merely state "Patient will return in 4 to 6 weeks." The notes further indicate that the patient continued to have mild disturbance in sleep, appetite, concentration and energy and ongoing marital discord. Patient #1 canceled her October 28, 1997 therapy session with the Respondent without providing a reason.

The Respondent claims that she spoke to patient #1 on the phone prior to commencing individual social work services for patient #2 and received the permission of patient #1 to proceed. According to the Respondent, patient #1 stated that she was feeling great and did not think that she would need the Respondent's services in the future. Patient #1 denies that the Respondent called her and testified that she did not know her husband was receiving services from the Respondent until May 1998.

The Respondent did not document a telephone call to patient #1 in either the records of patient #1 or patient #2. If such a phone call did occur as described by the Respondent, it would have been extremely important to have fully documented it to show that patient #1 waived any conflict of interest and considered her therapy to be completed. In the absence of such documentation, the Board gives greater weight to the testimony of the patient. The Respondent's patient-therapist relationship with patient #1 did not clearly terminate until September 24, 1998.

2. Patient #2

The Respondent's last progress note in the patient's record, dated April 21, 1998, states that the patient's stress level is still high due to his company not being sold yet. With respect to a return visit, Respondent wrote "will call if necessary." On April 30, 1998, patient #2 wrote to the Respondent. The Respondent asserts that with this letter, patient #2 terminated the patient-therapist relationship. However, the intent of this letter is unclear. Patient #2 refers to having met the Respondent's husband and having socialized with him. The Respondent thanks the Respondent for her services and states that he feels she would refer him if he needed more therapy. While this letter was placed in the patient's record, the Respondent does not make any notes of her own in the record recording termination of the patient-therapist relationship.

Once again, the first time that the Respondent makes any notation of her own in this patient's record regarding termination is when she includes a copy of the letter she provided to patient #2 on September 24, 1998. As noted earlier, the Respondent told the patient that she was removing herself from further professional therapeutic relationship with the patient due to changes in their associations which negate her ability to provide objective professional guidance and insight. The letter does not state that the relationship had previously been terminated. The preponderance of the evidence established that the Respondent did not terminate

her professional relationship with this patient until September 24, 1998.

C. Respondent's Inappropriate Dual Relationships With Patient #2

The purchase of a boiler and power washing services from patient #2 created a business relationship, which constituted a dual relationship between the Respondent and patient #2. Although the Respondent's husband handled all of the details of these purchases, the Respondent benefitted from the purchases and helped pay for them. She was aware that the purchases were being made. The Respondent acknowledged that she was uncomfortable with the purchases but felt that she was unable to tell her husband not to make them without breaching patient confidentiality. Based on this record, the Board believes that the Respondent could have advised her husband to purchase these items from someone else without breaching patient confidentiality.

Nevertheless, if only these two purchases were before the Board, it is unlikely that any disciplinary sanction would be imposed. However, these two purchases undoubtedly led to the third business relationship with patient #2. The Respondent demonstrated a complete lack of professional judgment when she and her husband hired patient #2 to perform extensive remodeling services in their home.

As pointed out by the state's expert witness, this business relationship required patient #2 to be in the Respondent's home during evening and weekend hours when the Respondent was also present. It allowed patient #2 to observe and interact with the Respondent and her family in their intimate home environment. It led directly to the formation of another improper dual relationship, a close friendship between the Respondent and patient #2.

In addition, the contract to remodel the Respondent's house required patient #2 to spend many additional hours away from his own wife and child, which the Respondent knew was a significant concern to patient #1 and a source of stress in their marriage. She had this knowledge as a direct result of the therapy services she had provided to these patients.

Given these circumstances, even if the patient-therapist relationship had been terminated by patient #2 on April 30, 1998, as claimed by the Respondent, it would still have been unethical for the Respondent to hire him for extensive remodeling services a

few months later. The NASW Code of Ethics prohibits dual relationships with clients or former clients in which there is a risk of exploitation or potential harm to the client. (State Exhibit M, section 1.06(c)) This dual relationship clearly risked exploitation and potential harm to both patient #2 and patient #1.

The Respondent claimed that she called patient #1 before hiring patient #2 to be sure that the arrangement would be all right with her. However, the Respondent also failed to document this telephone call in the record of either patient. Even if such a telephone call had been made, the Respondent cannot abdicate her responsibility to the patient. The power differential in the relationship clearly could have inhibited patient #1 from voicing any objections to the arrangement.

The Respondent was the licensed professional who should have been mindful of her own obligations, her power as the therapist, and the risks inherent in this business arrangement. The Respondent argued that if patient #2 had not been working for her, he would have been working for someone else. If he had been working for someone else, there may still have been stress on his marital relationship, but it would not have been compounded by the dual relationship with the Respondent.

The Board acknowledges that the recollections and perceptions of patient #1 may not be entirely accurate due to her emotional involvement and anger at both patient #2 and the Respondent. However, the same can be said of the perceptions and recollections of the Respondent and patient #2. By her testimony, it is clear that the Respondent has become emotionally involved and is not objective in her observations or assessments. Moreover, by engaging in inappropriate dual relationships with patient #2, the Respondent gave patient #1 reason to believe that the Respondent was a factor in the marital dissolution.

The Respondent's extremely poor judgment in hiring patient #2 to remodel her home was compounded when she embarked on a close social relationship with him: exchanging labor on each other's homes and socializing as families. After the marital separations, this social relationship became even more intimate and included patient #2 keeping his dogs at the Respondent's home which required daily visits; attendance at a high school and family reunion; and spending the night at each other's homes. There is no question that a close personal relationship of this type and magnitude violated the prohibition of dual relationships with clients or former clients. A relationship of this nature clearly posed a risk of exploitation or potential harm to the client(s).

The Respondent knew by March 1999 that her relationship with patient #2 was deeply disturbing and hurtful to patient #1. Despite this, the Respondent has continued the relationship. In addition, the Board also believes that patient #2 has been harmed by the dual relationships. In her interactions with patients #1 and #2, the Respondent has failed in her duty to put the interests of her patients ahead of her own personal interests.

It is apparent that as a result of her dual relationships with patient #2, the Respondent completely lost her objectivity with both of these patients. This was evident by her actions and by her testimony at the hearing. For example:

a. The Respondent failed to recognize or acknowledge that it was inappropriate to attempt to hypnotize patient #2 for any reason while he was working in her home. The Board does not accept the Respondent's explanations for the hypnosis as just "for fun". The Respondent received formal training in hypnosis and uses it on occasion in her social work practice. It was completely inappropriate for her to hypnotize or attempt to hypnotize a patient or a former patient while he was working in her home.

b. The Respondent attacked the credibility of patient #1 by repeatedly referring to her as a person with an Axis II personality who had "hidden agendas." The Respondent conceded that neither her records nor the records of patient #1's psychiatrist document any Axis II diagnosis for patient #1. This characterization of the patient without any supporting diagnosis was extremely unprofessional and self serving.

c. The Respondent and several of her witnesses established that the Respondent is normally extremely conservative in her approach to any dual relationship with a patient or former patient. The Respondent could not explain why her approach to patients #1 and #2 was so dramatically different from her usual approach.

d. The Respondent conceded that she did not consult with any other professional concerning the details of her dual relationships with these two patients, and her ethical obligation and responsibilities to them.

e. The Respondent claimed that the dual relationships in this case, if they existed, were unavoidable. The Respondent could have avoided hiring patient #2 to provide remodeling services in her home. The Respondent could have refused to engage in a personal friendship with either patient,

consistent with her past practice. The Respondent could have ended her personal relationship with patient #2 when it became apparent that the relationship deeply disturbed patient #1. Instead, the Respondent dismissed the concerns of patient #1 as fabricated or unreasonable.

II. COUNT II

Count II charged the Respondent with exercising undue influence on a patient or client.

645 IAC 280.213(3)"f" provides that a licensee shall not exercise undue influence on any client/patient or student, including the promotion of the sales of services, goods, appliances or drugs in a manner that will exploit the client/patient or student, for the financial gain or personal gratification of the licensee or of a third party.

While the facts of this case include some elements of undue influence, particularly with respect to the unequal power relationship between the Respondent and her two patients, the Board believes that the charges and findings under Count I adequately addresses the Respondent's ethical violations and more closely corresponds to the specific facts of this case. The Board makes no finding of violation on Count II.

III. SANCTION

The Board is authorized to impose the following disciplinary sanctions:

1. Revocation of license.
2. Suspension of license until further order of the board or for a specific period.
3. Prohibit permanently, until further order of the board, or for a specific period, the engaging in specified procedures, methods, or acts.
4. Probation.
5. Require additional education or training.
6. Require a reexamination.
7. Order a physical or mental evaluation, or order alcohol and drug screening within a time specified by the board.
8. Impose civil penalties not to exceed \$1000.
9. Issue a citation and warning.
10. Such other sanctions allowed by law as may be appropriate.

The following factors may be considered by the board in determining the nature and severity of the disciplinary sanction to be imposed:

1. The relative serious nature of the violation as it relates to assuring the citizens of this state a high standard of professional care.
2. The facts of the particular violation.
3. Any extenuating facts or other countervailing considerations.
4. The number of prior violations or complaints.
5. The seriousness of prior violations or complaints.
6. Whether remedial action has been taken.
7. Such other factors as may reflect upon the competency, ethical standards, and professional conduct of the licensee.

645 IAC 13.2.

The Board was disturbed by the Respondent's complete lack of insight and perspective concerning her inappropriate dual relationships and their detrimental effect on these two patients. She has taken no responsibility for her actions or the impact that they have had on the patients who relied on her professional judgement. She had numerous opportunities along the way to change or correct her course of action with respect to these two patients but failed to do so.

The Respondent admitted that the initial purchases of goods and services from patient #2 made her uncomfortable, but asserts that she did not know how to avoid them without violating ethical bounds. Despite her own misgivings about those purchases which should have alerted her to the conflict of interest, the Respondent subsequently allowed patient #2 to be hired to perform extensive remodeling work in her home during the hours when she and her family were also present in the home. The Respondent then embarked on a social relationship, initially with both patients, which later became exclusively a very close social relationship with patient #2. The close relationship with patient #2 is ongoing, and continues to cause harm and distress to patient #1.

Given the nature and seriousness of these ethical violations and the Respondent's apparent inability to recognize them, the Board must take aggressive disciplinary action, both for the protection of these two patients and for the protection of the public.

ORDER

IT IS THEREFORE ORDERED, that the social work license issued to Deborah C. Guffy, license no. 304-00741, is hereby SUSPENDED for an indefinite period. IT IS FURTHER ORDERED, that the suspension shall be immediately STAYED, subject to the Respondent's compliance with the following conditions:

1. The Respondent shall immediately terminate her social relationship with patient #2.

2. Within six (6) weeks of the issuance of this order, the Respondent shall submit to the Board the names of three licensed independent social workers (LISW) with whom she has no other business, personal, financial, or educational relationships. The Board will approve one of the three to serve as a practice supervisor for the Respondent. The approved supervisor will be provided a copy of the Board's Findings of Fact, Conclusions of Law, Decision and Order in this case.

The Respondent shall have one (1) hour of supervision for every fifteen (15) hours of patient contact. The supervisor shall review the Respondent's records for any similar situations to those presented in this case, with particular attention to all ethical issues, including boundaries and dual relationships. The supervisor shall provide monthly written reports to the Board indicating the Respondent's compliance or any lack of compliance with ethical practice. All costs associated with the supervision shall be the responsibility of the Respondent.

3. Within six (6) weeks of the issuance of this order, the Respondent shall submit the names of three licensed mental health professionals with whom she has no other business, personal, financial, or educational relationships. The Board will approve one of the three persons named by the Respondent. The approved licensed mental health professional will be provided a copy of the Board's Findings of Fact, Conclusions of Law, Decision and Order in this case. The Respondent shall immediately seek evaluation and assessment from the approved licensed mental health professional for any personal issues that may have clouded her professional judgement in this case. The evaluation, assessment, and any recommendations for treatment shall be submitted for Board review. The Respondent shall comply with any treatment recommended as a result of the evaluation and assessment.

4. After six (6) months of social work practice under supervision, the Respondent shall make a personal appearance before the Board.

The Respondent shall be required to comply with these conditions until further order of the Board. Failure to fully comply with these conditions will cause the Board to revoke the stay and impose the suspension of the Respondent's license.

IT IS FURTHER ORDERED, pursuant to Iowa Code section 272C.6 that the Respondent shall pay \$75.00 for fees associated with the disciplinary hearing within thirty (30) days of receipt of this decision. Any appeal to the district court from disciplinary action of the board shall be taken within thirty (30) days from the issuance of the decision by the board. It is not necessary to request a rehearing before the board to appeal to district court. 645 IAC 11.25. If an appeal is filed in district court, any costs incurred for the transcription of the record shall be paid by the Respondent.

Dated this 30th day of December, 2000.


Jennifer Lock Oman, L.I.S.W.
Vice-Chairperson
Iowa Board of Social Work Examiners

cc: Heather Adams
Assistant Attorney General

Connie Diekema
Attorney for Respondent

2000

BEFORE THE IOWA BOARD OF
SOCIAL WORK EXAMINERS

IN THE MATTER OF:	)	CASE NUMBER: 99-14
	)	
DEBORAH C. GUFFY,	)	STATEMENT OF CHARGES
	)	
Respondent.	)	

TO THE ABOVE NAMED RESPONDENT:

The Iowa Board of Social Work Examiners has jurisdiction of this matter pursuant to Iowa Code chapters 17A, 154C, and 272C (1997).

Licenses issued by the Board are subject to the laws of the state of Iowa and to the administrative rules of the Board.

Respondent Deborah C. Guffy was at all times during the following events a licensed independent social worker. Respondent was issued license number 304-00741 to practice as a social worker in the state of Iowa on July 16, 1991.

Respondent's current address as reported to the Board is 450 29th Street, Des Moines, Iowa, 50312.

COUNT I

Respondent is charged pursuant to Iowa Code section 147.55(3) and 645 Iowa Administrative Code 280.212(3) with engaging in unethical conduct in the practice of social work by engaging in a dual relationship with a patient or client.

COUNT II

Respondent is charged pursuant to 645 Iowa Administrative Code 280.213(3)(f) with exercising undue influence on a patient or client.

CIRCUMSTANCES

1. Respondent provided social work services to patient # 1 from March of 1996 to October of 1997. Respondent provided social work services to patient # 2 from March of 1996 to approximately August of 1998. At the time these services were provided, patient # 1 and patient # 2 were husband and wife.

2. Respondent provided individual therapy to both patient # 1 and patient # 2. Respondent also conducted numerous joint therapy sessions with both patient # 1 and patient # 2 present.

3. In October of 1997, Respondent and her husband purchased a boiler from patient # 2 for use at their residence.

4. In May of 1998, Respondent and her husband hired patient # 2 to clean the exterior of their residence.

5. In July of 1998, Respondent and her husband hired patient # 2 to remodel the kitchen of their residence. The remodeling required patient # 2 to spend numerous hours in Respondent's residence, including evening and weekend hours.

6. In July of 1998, Respondent provided hypno-therapy services to patient # 2 in her home.

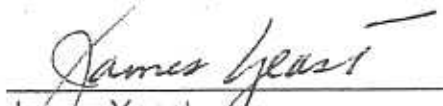
7. In August of 1998, Respondent and her husband assisted patient # 1 and patient # 2 with a remodeling project at the residence of patients # 1 and # 2. In August of 1998, Respondent and her husband and patient # 1 and patient # 2 socialized together.

8. Respondent engaged in an inappropriate relationship with patient # 2.

FINDING OF PROBABLE CAUSE

On January 21, 2000, the Iowa Board of Social Work Examiners found probable cause to file this Statement of Charges and to order that a hearing be set in this case.

Executed this 11th day of March, 2000.



James Yeast
Iowa Board of Social Work Examiners
Board Chairperson

STATE OF IOWA
BEFORE THE IOWA BOARD OF SOCIAL WORK EXAMINERS

IN THE MATTER OF:	)	CASE NO. 99-014
	)	
DEBORAH C. GUFFY	)	NOTICE OF HEARING
License No. 304-00741	)	
	)	
RESPONDENT	)	

YOU ARE HEREBY NOTIFIED that on January 21, 2000, the Iowa Board of Social Work Examiners found probable cause to file a Statement of Charges against you. If any of the allegations against you are founded, the Board has authority to take disciplinary action against you under Iowa Code chapters 17A, 154C, and 272C (1997) and 645 Iowa Administrative Code chapter 9, 11, 13, and 280.212. A copy of the Statement of Charges is attached, and sets forth the particular statutes and rules which you are alleged to have violated, and further provides a short and plain statement of the matters asserted.

IT IS HEREBY ORDERED that a disciplinary contested case hearing be held upon the Statement of Charges on Monday, May 8, 2000, before the Board. The hearing shall begin at 9:00 a.m., and shall be held in the Lucas State Office Building-5th Floor Professional Licensure Board Room, 321 E. 12th Street, Des Moines, Iowa. The Board shall serve as presiding officer, but the Board may request an Administrative Law Judge make initial rulings on pre-hearing matters, and be present to assist and advise the Board at hearing.

Within 20 days of the date you are served with the Statement of Charges and Notice of Hearing, you are required by 645 IAC 11.12 to file an answer to the charges. In that answer, you should also state whether you will require an adjustment of the date and time of the hearing.

At hearing, you may appear personally and be represented by counsel at your own expense. You will be allowed the opportunity to respond to the charges against you. The procedural rules governing the conduct of the hearing are found at 645 IAC chapter 11.

The office of the Attorney General is responsible for representing the public interest (the State) in this proceeding. Copies of all pleadings filed with the Board should be provided to counsel for the State at the following address:

Heather Adams
Assistant Attorney General
Iowa Attorney General's Office
2nd Floor, Hoover State Office Building
Des Moines, IA 50319
Phone (515) 281-3441

If you fail to appear at hearing, the Board may enter a default decision or proceed with the hearing and render a decision in your absence, in accordance with Iowa Code section 17A.12(3) and 645 IAC 11.21.

This matter may be resolved by settlement agreement. The procedural rules governing the Board's settlement process are found at 645 IAC chapter 12. If you are interested in pursuing settlement of this matter, please contact Heather Adams, Assistant Attorney General, at the above telephone number.

Dated this 15th day of March, 2000.

A handwritten signature in cursive script, appearing to read "Roxanne Sparks by [initials]".

Roxanne Sparks, Board Administrator
Iowa Board of Social Work Examiners